

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 08

Age: 58 years old

Sex: Female

MRN: 000-TEST-08

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 4 days

****PRIMARY DIAGNOSIS****

Congestive Heart Failure with reduced ejection fraction (HFrEF), NYHA Class III-IV

****SECONDARY DIAGNOSES****

- Obstructive Sleep Apnea (OSA), moderate-severe, on CPAP therapy
- Hypertension
- Type 2 Diabetes Mellitus
- Hyperlipidemia
- History of Atrial Fibrillation (paroxysmal)

****HOSPITAL COURSE****

Patient 08 presented to Metro Medical Center with acute decompensated heart failure manifesting as dyspnea on exertion (DOE), orthopnea, and lower extremity edema. Initial

presentation included elevated BNP of 1,847 pg/mL. Transthoracic echocardiography confirmed severely reduced left ventricular ejection fraction (LVEF) of 22% with global hypokinesis. Patient was admitted for IV diuretic therapy and medical optimization.

During hospitalization:

- Received IV furosemide 40 mg q.d. x 3 days with transition to oral diuretics
- Initiated sacubitril/valsartan (ARNI) for neurohormonal antagonism
- Continued home beta-blocker with dose adjustment
- Diuresed 6.2 liters total; orthopnea resolved
- SpO2 maintained 94–98% on room air with CPAP at night (settings: 10 cm H₂O)
- Discharged when euvolemic with stable renal function (Cr 1.1)

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
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Sacubitril/Valsartan	97/103 mg	BID	Indefinite
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Furosemide	40 mg	q.d. morning	Indefinite
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Metoprolol succinate (extended-release)	150 mg	q.d.	Indefinite
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Spironolactone	25 mg	q.d.	Indefinite
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Atorvastatin	80 mg	q.d. evening	Indefinite
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Lisinopril (continue)	10 mg	q.d.	Indefinite
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Metformin	1000 mg	BID with meals	Indefinite
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Aspirin	81 mg	q.d.	Indefinite
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Omeprazole	20 mg	q.d. morning	PRN GI symptoms
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****ACTIVITY & WEIGHT MANAGEMENT****

- Ambulation ad lib; avoid strenuous exertion and heavy lifting (>10 lbs)
- ****Daily weight monitoring:**** weigh self each morning after void; notify cardiologist if weight gain >2–3 lbs in 24 hours or >5 lbs in 1 week
- Gradual return to light ADLs; cardiac rehab referral provided

****DIETARY RESTRICTIONS****

- Strict sodium restriction: <2 grams/day
- Fluid restriction: 1.5 liters/day
- Continue diabetic diet; avoid high-sodium, processed foods

****FOLLOW-UP APPOINTMENTS****

1. Cardiology clinic — in 2 weeks (Dr. R. Patel, Metro Medical Cardiology)
2. Primary Care — in 1 week
3. Sleep Medicine —